



MORIDONE TABLET

Description: White, round, biconvex and top-scored tablet. Diameter = 6.00mm

Content: Each tablet contains domperidone 10mg

Therapeutic Indications:

Domperidone is indicated for the relief of the symptoms of nausea and vomiting.

This includes:

- Nausea and vomiting of functional, organic, infectious or dietary origin.
- Nausea and vomiting induced by:
 - radiotherapy or drug therapy.
 - dopamine agonists (such as L-dopa and bromocriptine) used in the treatment of Parkinson's disease.

Pharmacology:

Domperidone stimulates gastrointestinal motility and issued in the management of some forms of nausea and vomiting. It is a dopamine antagonist but is less likely than metoclopramide to produce extrapyramidal reactions, plasma prolactin concentrations may be increased.

Domperidone undergoes extensive first pass hepatic and gut wall metabolism which results in an oral bioavailability of 13% to 17%. Total plasma clearance is approximately 700ml/min. The elimination half-life following oral administration is approximately 14 hours.

Adverse effects / Side Effects:

Side effects include mild, abdominal cramps which occur rarely and allergic reactions such as rash and urticaria. Plasma prolactin concentrations may also be increased which may lead to galactorrhoea or gynaecomastia. The use of domperidone may be associated with ventricular arrhythmias and sudden cardiac death. (Frequency unknown). See "Warnings and Precautions-cardiovascular effects"

Postmarketing:

Cardiac Disorders
Frequency: Very rare
Ventricular arrhythmias, QTc prolongation, Torsade de Pointes, Sudden cardiac death (see Warnings and Precautions)

Warnings and Precautions

Cardiovascular effects

Domperidone has been associated with prolongation of the QT interval on the electrocardiogram. During post-marketing surveillance, there have been very rare cases of QT-prolongation and torsades de pointes in patients taking domperidone. These reports included patients with confounding risk factors, electrolyte abnormalities and concomitant treatment which may have been contributing factors (see Adverse Reactions).

Epidemiological studies showed that domperidone was associated with an increased risk of serious ventricular arrhythmias or sudden cardiac death (see Adverse Reactions). A higher risk was observed in patients older than 60 years, patients taking daily doses greater than 30mg, and patients concurrently taking QT-prolonging drugs or CYP3A4 inhibitors.

Domperidone should be used at the lowest effective dose in adults and children.

Domperidone is contraindicated in patients with known existing prolongation of cardiac conduction intervals, particularly QTc, in patients with significant electrolyte disturbances (hypokalaemia, hyperkalaemia, hypomagnesaemia), or bradycardia, or in patients with underlying cardiac diseases such as congestive heart failure due to increased risk of ventricular arrhythmia (see Contraindications).

Electrolyte disturbances (hypokalaemia, hyperkalaemia, hypomagnesaemia) or bradycardia are known to be conditions increasing the proarrhythmic risk.

Treatment with domperidone should be stopped if signs or symptoms occur that may be associated with cardiac arrhythmia, and the patients should consult their physician.

Patients should be advised to promptly report any cardiac symptoms.

Pregnancy & Lactation

As it is also excreted in breast milk, special precautions to lactating mother, pregnancy and infants.

Drug Interactions:

It is antagonized by anticholinergic drugs. Antacids or antisecretory agents should be taken after meals when taken concomitantly.

Contraindications:

Moridone Tablet is contraindicated in the following situations:



- Known hypersensitivity to domperidone or any of the excipients.
- Prolactin-releasing pituitary tumour (prolactinoma).
- In patients who have known existing prolongation of cardiac conduction intervals, particularly QTc, patients with significant electrolyte disturbances or underlying cardiac diseases such as congestive heart failure (see Warnings and Precautions).
- co-administration with QT-prolonging drugs
- co-administration with potent CYP3A4 inhibitors (regardless of their QT-prolonging effects)
- Whenever stimulation of gastric motility might be dangerous, e.g., in the presence of gastro-intestinal haemorrhage, mechanical obstruction or perforation.
- In patients with moderate or severe hepatic impairment.

Dosage and Administration

It is recommended to take Moridone Tablet 15-30 minutes before meals. If taken after meals, absorption of the drug is somewhat delayed.

Adults and adolescents ≥ 12 years and weighing ≥ 35 kg and children weighing ≥ 35 kg

The dose of Moridone Tablet should be the lowest effective dose for the individual situation (typically 30mg/ day) and can be increased if necessary to a maximum daily oral dose of 40mg.

Usually, the maximum treatment duration should not exceed one week for the treatment of acute nausea and vomiting. If nausea and vomiting persists for longer than one week, patients should consult their physician. For other indications, the initial duration of treatment is up to four weeks. If treatment exceeds four weeks, patients should be reevaluated and the need for continued treatment reassessed.

Formulation (domperidone per unit)	Dosage	Maximum dose per day
Film-coated tablet (10mg/ tablet)	Tablet three to four times per day	40mg (4X10mg tablet)
Oral suspension (1mg/ ml)	10ml three to four times per day	40mg (40ml of 1mg/ml oral suspension)

Neonates, infants and children <12 years of age and weighing <35 kg, and adults and adolescents weighing <35 kg

The dose of Moridone Tablet should be the lowest effective dose. The total daily dose is dependent on weight (see table below). Since metabolic functions and the blood-brain barrier are not fully developed in the first months of life, the risk of neurological side effects is higher in young children. Overdosing may cause nervous system disorders in children. The dose should be determined accurately based on body weight and not exceed the recommended maximum individual and daily dose in neonates, infants, toddlers and children.

Usually, the maximum treatment duration should not exceed one week for the treatment of acute nausea and vomiting. For other indications, the initial duration of treatment is up to four weeks. If treatment exceeds four weeks, patients should be reevaluated and the need for continued treatment reassessed. Film-coated tablets and orodispersible tablets are unsuitable for use in children, adults and adolescents weighing less than 35kg. Suppositories are unsuitable for use in children.

Formulation (domperidone per unit)	Dosage	Maximum dose per day
Oral Suspension (1mg/ ml)	0.25mg/kg three to four times per day	1mg/kg but no more than 35 ml (35mg)

Renal impairment

Since the elimination half-life of domperidone is prolonged in severe renal impairment (Serum creatinine >6 mg/ 100ml, i.e. >0.6 mmol/L), the dosing frequency of Moridone Tablet should be reduced to once or twice daily, depending on the severity of the impairment, and the dose may need to be reduced. Patients with severe renal impairment should be reviewed regularly.

Hepatic impairment

Moridone Tablet is contraindicated for patients with moderate (Child-Pugh 7 to 9) or severe (Child-Pugh >9) hepatic impairment. Dose adjustment is not required for patients with mild (Child-Pugh 5 to 6) hepatic impairment.

Treatment of overdosage

Drowsiness, disorientation, dystonic reactions e.g. oculogyric crisis, opisthotonus, torticollis, trismus, a tetanic like reactions are readily reversed by an antihistamine such as diphenhydramine or an antimuscarinic such as benztropine or activated charcoal.

Storage Conditions : Store in a cool, dry place below 30°C. Protect from light. Keep away from children

Presentation : Tablet X 100 X 10's / blister

Shelf life : 3 years

Manufactured & : PRIME PHARMACEUTICAL SDN. BHD.

Distributed by : 1505, Lrg Perusahaan Utama 1, Tmn Perindustrian Bkt Tengah, 14000 Bukit Mertajam, Pg. Malaysia.